



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : حنان محمد محمود متولي
المحافظة : الجيزة
الحى / المدينة : الوراق
رقم البطاقة : 26611030101445 رقم التليفون : 01112828274
العنوان : =
العمر (تاريخ الميلاد) : 03-11-1966 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- / د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: حنان محمد محمود متول National ID: 26611030101445	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

حنان
محمد محمود متولى

١ ش ترعة السواحل - جزيرة محمد
الوراقى - الجيزة

٢٦٦١١٠٣٠١٠١٤٤٥

IP5114660

٢٠٢٠/١٠ ٢٦٦١١٠٣٠١٠١٤٤٥

ربة منزل

أنثى
متنصر محمد أحمد البهنسى

البطاقة سارية حتى ٢٠٢٧/١٠/٢٤

١



وزارة الصحة
المملكة العربية السعودية

السرطان
MDT
2022

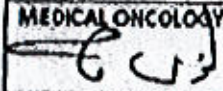
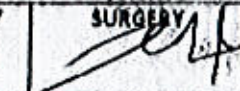
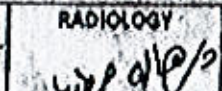
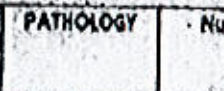
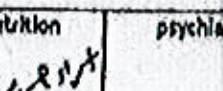
MDT FORM 22 / 8 / 2022 GC

Name	فاطمة عبد الله	MRN	Y517-5.00
Age	78 y	ID	580.75.1.902
Menopausal status	post.	Phone	1.00.110.4.9
NRSSCORE		Marital status	
BMI		Family history	
Co morbidity	DM, HPTN, Stroke twice, Vertebral fixation 10y ago		

Clinical Data			
Rx:	mixed IDC & ILC GII		
ER:	PR:	HER2:	Ki67:
8/8	8/8	+3	40%
Sonomammography:			
Metastasis Workup:			
			
pT ₃ N ₁ M ₀ (2/22)			

Recommendations: Right MRM done 17.4.2022
she has multiple Comorbidities

MDT OUTCOME & PLAN Rx - adjuvant Herceptin SC x 1 year
Tamoxifen x 5 years

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	psychiatric
					

Patient Name : الأستاذة / حنان محمد محمود متولى
Patient No. : 26611030101445
Path No. : 1088/22
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13008730
Request Date : 20/04/2022
Verification Date : 08/05/2022
Financial Category : نفقة الدولة - اورام دار السلام
Department : In Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Right breast cancer, (post neoadjuvant therapy)

Nature of the specimen:

Right WLE & SLNs

Gross Examination:

Two specimens were received submitted as:

1-Right WLE measuring 8x6x3 cm, with skin covering; 6x2 cm. Cut section revealed an ill-defined firm greyish white firm mass measuring 4x2x2 cm, located 2 cm, 0.7 cm, 1 cm, 2cm, 2 cm, 0.5cm away from lateral, medial, deep, superior, inferior, superficial resection margin respectively.

2- Sentinel lymph node: Fibrofatty tissue piece measured 3x2.5cm entangling 4 nodules ranging in size from 0.5 cm to 1 cm.

• Frozen section: Free margins & Free sentinel lymph nodes

Microscopic examination:

- Remnants of tumor tissue
- Partial pathological response
- Angioinvasion: No
- Ductal carcinoma in situ: Focal
- Lymph node involvement: Free (0/4)
- Resection Margins: Free

Diagnosis:

Right breast cancer, Right WLE & SLN (post neoadjuvant);

Remnants of tumor tissue

Partial pathological response

Verified by :

Dr.Marwa mahmoud

Printing Date : 10/05/2022 09:36:05

Page 1 of 2

Printing By : marwa mohamed y

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذة / حنان محمد محمود متولى	Request No.	: 13008730
Patient No.	: 26611030101445	Request Date	: 20/04/2022
Path No.	: 1088/22	Verification Date	: 08/05/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 56 Y	Department	: In Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Free resection margins.
Free covering skin.
Free sentinel lymph nodes (0/4)
Stage: ypT2 ypN0

Prof Dr. Gamal Abd Elaty
Dr. Israa Elgendy

Verified by :
Dr.Marwa mahmoud

Printing By : marwa mohamed youss

Printing Date : 10/05/2022 09:36:05

Page 2 of 2



National Women's Health Initiative
Pathology Lab

معمل باثولوجي المبادرة القومية لصحة المرأة

Pathology Report

Name: هنان محمد محمود متولى

Age: 55 years.

Clinical data: Right breast.

Specimen type: Tru cut biopsy.

Date (Received): 11 /7/2021

Date(reported): 11 /7/2021

Lab. Number: 1493/21

Hospital Number: 51 صبا

Gross Examination:

Four grayish white tissue cores, 0.7 cm up to 1.5 cm, totally submitted.

- Procedure: Tru-cut biopsy.
- Localization: Right breast
- Tumor extent
 - Number: N/A
 - Size: N/A
 - Skin involvement: N/A
- Distance from margins: N/A
- Lymph nodes number and size: N/A
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): G II
- Angioinvasion: N/A
- Ductal carcinoma in situ: N/A
- Tumor extent
 - Number: N/A

- Size: N/A
- Skin involvement: N/A
- Lymph node involvement: N/A
- Resection Margins: N/A
 - Free /DCIS/ invasive carcinoma
- Additional pathologic findings: N/A

Diagnosis:

Right breast mass; Tru-cut biopsy:

Invasive duct carcinoma, grade II

Biomarkers study:

ER: Strong nuclear reaction in more than 70% of tumour cells. Positive, score (8/8)

PR: Strong nuclear reaction in 60% of tumour cells. Positive, score (3)

Her 2: No membrane reaction. Negative, score 0

Ki67: 15%

Prof. Dr. Gamal Abdel- Atty

Dr/ Walaa Ghonaim.

Dr. Abdulrahman Ibrahim

Name : Hani M. Kamel Date : 18-04-2021

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest

- Right breast: UIQ irregular speculated mass lesion with central areas of degeneration measuring 2.5 X 2.3 cm.
- No sizable pulmonary nodules, masses or consolidations.
- Middle and lingular lobes atelectatic bands.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi.

2. Abdomen and pelvis

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows no definite lytic or sclerotic lesions.



Patient Name: Hanan Mohamed Mahmoud Metwally	Date : 04/03/2023
Exam : MAMMOGRAPHY BOTH	Age : 56 Year(s)
Medical ID: 3337/2021	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in right side

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C).
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast UIQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UIQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2).

Yours truly

Dr.Wafaa Ali

Signature Temp

04/03/2023 1:25 PM

Name : Munon Mohammed Date : 18-11-17

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
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Patient Name: Hanan Mohaned Mahmoud Metwally	Date : 27/07/2025
Exam : Left upper limb venous doppler	Age : 58 Year(s)
Medical ID	Pat. ID: 29500

Duplex Examination of the venous system of left upper limb revealed: -

- **The left IJV** attenuated over the port-A-Cath filled with adherent echogenic thrombus with no color filling or doppler wave denoting chronic thrombosis
- The subclavian, axillary, radial & ulnar veins of the examined upper limb all are patent, totally compressible with normal color filling & preserved respiratory variation.
- Patent cephalic & basilic veins along their course

OPINION: -

→ ***Left IJV chronic thrombosis***

Signature Dr . Nermien Ramadan
PM 3:54 2025/07/27